

St. Mary's Medical Center

PATIENT INTAKE FORM (EMERGENCY DEPARTMENT)

Date of Visit: 07/28/2026

Arrival Time: 08:15 via EMS

PATIENT INFORMATION:

First Name: Harold

Last Name: Brooks

Date of Birth: 04/30/1948 (Age 78)

Sex: Male

MRN: MRN-2025-0112

Phone: (555) 402-6611

Address: 17 Prairie View Court, Springfield, IL 62711

Insurance: Medicare + Aetna Supplement

Policy Number: MCR-778-1042

EMERGENCY CONTACT:

Name: Nancy Brooks (Daughter)

Phone: (555) 402-7788

CHIEF COMPLAINT:

Fever, confusion, and decreased urination for 2 days

HISTORY OF PRESENT ILLNESS:

78-year-old male brought by family with 2 days of fever to 103.1 F at home, new confusion (non-verbal overnight per daughter), burning urination with foul-smelling urine, and markedly reduced urine output. History of recurrent UTIs; most recent course completed 6 weeks ago. Found collapsed attempting to stand this morning.

PAST MEDICAL HISTORY:

1. Benign prostatic hyperplasia
2. Recurrent urinary tract infections (3 in past year)
3. Chronic kidney disease stage 3a
4. Atrial fibrillation on apixaban
5. Hearing impairment (bilateral aids)

SURGICAL HISTORY:

- TURP (2019)

- Inguinal hernia repair (2014)

HOME MEDICATIONS:

1. Tamsulosin 0.4 mg PO daily
2. Apixaban 5 mg PO BID
3. Atorvastatin 40 mg PO daily
4. Nitrofurantoin 100 mg PO BID (suppressive)

ALLERGIES:

- Codeine (hives)

FAMILY HISTORY:

- Non-contributory per family

VITAL SIGNS ON ARRIVAL:

- Blood Pressure: 92/54 mmHg
- Heart Rate: 112 bpm, irregularly irregular
- Temperature: 102.9 F
- Respiratory Rate: 26 breaths/min
- O2 Saturation: 93% on room air
- Glucose fingerstick: 148 mg/dL

ED IMPRESSION:

Suspected urosepsis with hypotension and altered mental status. Two large-bore IVs placed, 30 mL/kg crystalloid initiated, blood and urine cultures drawn, stat labs and lactate ordered. Empiric broad-spectrum antibiotics administered within 40 minutes of arrival per sepsis protocol. Urology and ICU consulted.